

So you're thinking about dentistry

The short version — written for you, not about you.

Everything that matters, none of the bits written for your parents.

We scored twenty-eight careers on how exposed they are to AI.

General dentistry: **2.7 out of 10**. Oral surgery: **2.3**.

Among the safest work we measured anywhere. Entry-level software development is 8.1.

So the AI question is settled. **The question worth thinking about is the money**, and it's a bigger one than most people realise.

The 60-second version

- Dentistry is protected by all four of our factors at once. That's rare.
 - Average dental school debt is \$293,900 — and the spread between schools is over \$400,000.
 - The income model assumes you'll eventually **own a practice**. Employed dentists earn far less.
 - **Dentistry already ran this experiment once**, and the result is the most useful thing in this document.
 - And dental hygiene pays a median slightly above a nurse, with a fraction of the debt.
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The thing dentistry already proved

This is worth understanding properly, because it applies to every career you might consider.

Dental lab work was automated more than a decade before anyone was talking about AI.

Crowns, bridges and dentures used to be made by hand by skilled technicians. Real craft, learned over years, genuinely artistic. Then CAD/CAM arrived — digital scanning, computer design, milling machines.

Much of that work moved from a bench to a screen and a machine, and far fewer people were needed.

Now look at what happened next:

The profession was completely fine. Dentists still practise. Patients still get crowns. Demand didn't drop.

But a whole tier of skilled work contracted permanently, and the people in it mostly didn't move up — they moved out.

That's the lesson: automation doesn't usually kill a profession. It removes a tier of it. And which tier you're in matters more than which profession you pick.

Two things follow from that, and they're worth carrying everywhere:

The tier that goes first is the one with structured inputs and an output a machine can produce — a crown from a scan, a document from a template, an image from a brief.

The tier that survives is the one with the patient, the client, or the accountability directly in front of it.

And nobody outside dentistry noticed the lab tier disappear. The same thing is happening right now inside law, accounting and design, and from the outside it looks identical: the profession seems healthy, and one layer of it has quietly stopped being a career.

The scores

TRACK	SCORE
Oral & maxillofacial surgery	2.3
General dentistry	2.7
Dental hygiene	3.0
Orthodontics	3.8
Dental laboratory technology	5.5

Same pattern as everywhere: **the further from the patient's mouth, the higher the exposure.**

Orthodontics is the fastest-moving track in the profession — aligner design, treatment planning and remote monitoring are genuinely automatable in a way that a filling isn't.

Why chairside work is so protected

You're doing fine motor work inside a wet, confined, moving space belonging to someone who may be frightened. That's among the hardest physical problems in medicine, not one of

the easier ones.

It's **comprehensively licensed**. Practice, prescribing and procedures are all legally reserved.

And dentistry runs on trust more than most healthcare, because a lot of patients are genuinely afraid. Trust is what gets people through the door and back again in six months.

Now the money

The debt is serious:

- Average dental school debt: **\$293,900** (ADA's own survey of dental school seniors)
- Public schools: **\$200,000–\$260,000**
- Private: around **\$321,000**
- The most expensive programmes: **\$625,000–\$700,000**

And here's the part that matters most:

- BLS puts the mean wage for general dentists at **\$166,810**
- ADA data puts median net income for practice owners closer to **\$239,000**

That gap is the whole story. Dentistry's income model assumes you'll eventually own a practice. If you stay employed, the numbers look considerably worse against that debt.

Worth knowing at seventeen rather than at thirty-two.

One more statistic that says something. ADA data indicates around **35% of dental school costs are paid without loans**, with another 17% covered by family. Dentistry draws disproportionately from families who can fund it — which means the "average debt" number understates things for anyone borrowing the whole amount.

The practical instruction: go to the cheapest accredited school you get into. The spread between programmes exceeds \$400,000 for exactly the same licence.

The alternative worth an actual hour

Dental hygiene, and the numbers are surprising:

- Median pay **\$98,100** — slightly above the registered nurse median of \$97,550

- **No night or weekend shifts** (unlike nursing)
- **Debt of \$30,000–\$80,000** — an order of magnitude below dental school
- **7% growth**, about 15,300 openings a year
- Scores **3.0** on our index, against 2.7 for dentistry

Two honest caveats, because this isn't free money:

About **one in three hygienists works part-time** — 28.8% under 32 hours a week, against 53.7% full-time. Annual salary figures assume full-time hours a lot of people don't work.

And benefits follow from that — health insurance and paid leave are far from universal in private dental practices, and often depend on full-time status.

There's also less of a ladder. Nursing has a longer advancement path; hygiene is fairly flat.

But: **two to three years and \$50,000** against **seven-plus years and \$293,900**. If what pulls you is clinical dental work rather than diagnosis, treatment planning and running a business, that's a comparison worth making properly.

What we won't soften

Your body takes a beating. Dentistry is done in sustained awkward postures, and neck, back, shoulder and wrist problems are a recognised occupational hazard that ends careers early.

Patient fear is constant. A lot of your patients are frightened, and managing that over and over is more emotionally demanding than people outside the job expect.

The business is a second job. Ownership is where the money is, and running a practice means staff, overheads, insurance negotiation and marketing — none of which dental school teaches well.

And it's repetitive by design. Same room, same chair, similar procedures. Some people find that calming. Others find it deadening.

The genuinely good news

Satisfaction is high: around 74% report career satisfaction, and 85% say they'd choose their specialty again — at the higher end of healthcare.

And the hours are better than most of medicine — around **36 hours a week** on average, with no routine nights or weekends in general practice.

That combination — protected career, high satisfaction, sane hours, immediate visible results — is genuinely rare.

Does this actually sound like you?

It suits people who:

- **Genuinely enjoy precise manual work** — this is testable, and it's the core of the job
- Are **comfortable around frightened people**, repeatedly
- **Want to fix something and see it fixed** the same day
- Are **interested in running a business**, or at least willing to be
- Can work in **confined, repetitive physical conditions** for decades

Harder if you want variety of setting, are uncomfortable causing discomfort — which is unavoidable — or have no interest in the commercial side, since that's where the income sits.

What actually matters when picking a school

- **Total cost. Calculate the debt-to-income ratio yourself.** The spread exceeds \$400,000 for the same licence.
 - **Accreditation** — non-negotiable
 - **How many procedures do students actually perform before graduating?** This varies enormously and determines how competent you are on day one.
 - **Is practice management compulsory?** It's where the income comes from and it's usually underweighted.
 - **Digital workflow training** — scanning, CAD/CAM, treatment planning
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Two things worth doing

Shadow in two very different practices — a private owner-operated one and a corporate/DSO one. The economics and the culture are genuinely different and it'll shape which you aim for.

Then find a dentist about five years qualified and ask directly about their loan payments and whether they'd pick the same school again. Awkward question, valuable answer.

One last thing

If you like the idea of precise hands-on healthcare with sane hours and visible results, dentistry is a genuinely good answer — and one of the best-protected careers we measured.

Just go in knowing two things: the money assumes you'll eventually own something, and where you study will affect your finances for twenty years.

And give dental hygiene a real hour of thought rather than dismissing it. For a lot of people it's closer to the job they were actually picturing.

If you want to go further — three things worth twenty minutes

- **Dentists — Occupational Outlook Handbook** — government data on pay and growth. Read the **Dental Hygienists** page straight after; that comparison is the most useful twenty minutes here.
 - **ADA Health Policy Institute** — the profession's own research on income, ownership and consolidation. Where the owner-versus-employee numbers come from.
 - **Average dental school debt** — school-by-school cost data. Read it before deciding where to apply, not after.
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There's a longer version behind this — full scores by track, how they were worked out, the financial picture in detail, what to ask a school, and where we might be wrong. Your parents have it.